

WHY INDIA NEEDS CESSATION SUPPORT

New research now offers hope to those who want to quit smoking but have failed. For decades, the most common answer to tobacco cessation was to put a hard stop or opt for nicotine-replacement therapy (NRT) in the form of gums, patches or sprays. If it led to emotional and physical withdrawal effects, there was medicine for that too. Today, the entire approach to cessation has changed. An October 2021 study on tobacco cessation by the University of Bristol published in the American journal *Addiction* recommended a combination approach using behavioural counselling, NRT and new modern drugs such as varenicline and bupropion. These drugs, in fact, have come into modern medical use only early this year, with the WHO characterising them as safe and effective for tobacco abstinence. The new medicines suppress the craving for nicotine without having to depend on any substitute and help manage withdrawal symptoms.

"In my experience, simply asking someone to quit cold turkey and giving NRT drugs hardly ever works," says Dr Prashant Chhajed, director of pulmonology at the Fortis Hiranandani Hospital in Vashi, Mumbai. "We need awareness, motivation, a thorough understanding of triggers, a psychology assessment, building social support and then quitting and medicine."

One reason why smokers find it difficult to give up smoking for good is their inability to control triggers and going into a relapse. For Nikhil Gupta, a 32-year-old MBA student in Noida, it was a late-night office party sometime in June. It had been four months since he had picked up a cigarette. On that particular day, he succumbed. He has been smoking almost half a pack daily since, reverting to a habit he thought he had gotten rid of. "This is the third time I have had a relapse," he says. "I don't know if I have the strength to stop again."

Since self-regulation is such an elusive beast, doctors now recommend that smokers seek professional support. "Cessation support can help addicts identify, understand and cope with triggers that cause a relapse," says Dr Vikrant Mohanty, head of the newly-inaugurated National Resource Centre for Oral Health and Tobacco Cessation at the Maulana Azad Institute of Dental Sciences (MAIDS) in Delhi, who has two decades of experience in the field. The government has mandated that a tobacco cessation clinic be set up in all government dental colleges in India. Dentists have been roped in for the exercise as India has over 281,000 registered practitioners and 315 dental colleges with 35,000 faculty members and 26,000 students each. Also, as Dr Mohanty points out, "A dentist can just peer into a mouth and know right away if someone is using tobacco, and since they meet patients multiple times, support and motivation to quit can be given over a long period."

NEW WAYS TO QUIT AN OLD HABIT

"Most people who try to quit," says Mumbai-based psychiatrist Dr Kedar Tilwe, "do so without understanding the addiction part of tobacco and are thus not prepared for how the body and mind will respond." It is a well-established fact that nicotine, a chemical found in tobacco, has a powerful and direct impact on dopamine release in the brain. Over time, the brain starts to seek tobacco as a means for continued stimulation. "Many," says Dr Tilwe, "start smoking to curb anxiety. But in the gaps that they are without tobacco, they start to have even more anxiety, triggering a clear dependency pattern." Unable to sustain the attempt to wean away from smoking, they restart after a few weeks or months, sometimes even years.

It is becoming increasingly clear that the mere desire to quit may not be enough, those who truly want to stop